NHS T.E.S.T. Framework

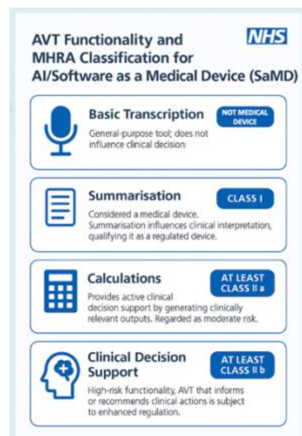
NHS T.E.S.T. ensures a fair and transparent selection process by applying consistent evaluation criteria across all vendors. This fosters healthy competition and enables the NHS to adopt the most effective technologies. If your ICS has already approved an AVT vendor using T.E.S.T., individual Trusts, PCNs, or Surgeries may not need to conduct separate assurance processes for each supplier and their solution. Liability from the choice of non-compliant technologies will rest locally.

Section A - **Platform stability, cybersecurity, and data assurance**

Assurance is assessed across 7 domains with 22 key requirements. Compliance with **EACH** requirement is a **mandatory minimum** to achieve certification and progress to Section B.

Domain	Requirement	Passed	Fail	Justification
Cybersecurity Requirements	1. Any software vendor must have NHS accreditations: DTAC (Digital Technology Assessment Criteria), DSPT, CyberEssentials Plus, CREST-approved penetration testing, and must be UK GDPR compliant.			
	- DTAC			
	- DSPT			
	- CyberEssentials Plus (CE Plus)			
	- CREST-approved penetration testing			
	- UK GDPR Compliant			
Data Protection Requirements	2. Safeguarding Patient Information is paramount. Patient data may be stored in a public cloud or vendor database only if: • The service complies with NHS Digital's Data Security and Protection Toolkit • End-to-end encryption is ensured • A Data Protection Impact Assessment (DPIA) is completed (ICO, DPIA Guidance).			

Domain	Requirement	Passed	Fail	Justification
	Storage outside the host EHR is discouraged but permitted if: • Patients are informed. • Data is stored in a Trusted Research Environment (TRE) or under an approved data-sharing agreement for research or analysis Data controllership needs to rest with the healthcare org not the developer (for direct care) or with the research sponsor (for research)			
	3. Deletion of Patient Data: Patient data from clinical sessions (e.g., immediate inference) should be automatically deleted unless legally or operationally required, in line with UK GDPR and DPA 2018 principles on data minimisation and storage limitation. This includes input data (such as voice files/recordings) and output data (such the Ambient scribe transcript) – that are involved in creating the Ambient summary . At the end of the clinician session, ideally only the Ambient Summary / note created should be saved in the host EHR Exceptions to Immediate Deletion: Temporary retention may be permitted for: •Error checking, audit trail, reconciliation, or technical troubleshooting. •Aggregated or anonymised data used for operational analysis or QA. Retention must remain proportionate and adhere to data security standards			
	The upcoming EU AI Act further regulates AI systems, above UK GDPR, based on their risk level. High-risk AI systems (which include healthcare applications) must comply with strict data governance and transparency rules. 4. Importantly, data used to train AI systems must adhere to high standards for quality, minimisation, and anonymisation to ensure privacy protection). ICO (UK Regulator Guidance) should be followed:			

Domain	Requirement	Passed	Fail	Justification
	https://ico.org.uk/for-organisations/uk-gdpr-guidance-and-resources/artificial-intelligence/			
	5. All servers for such systems must be physically located in Europe, preferably the UK, to meet UK GDPR and NHS SDPT requirements, ensuring compliance and to avoid additional legal complexities related to international data transfers. (The European Union and the UK have similar data protection standards under GDPR, so hosting servers within the EU or UK meets legal requirements). Data Transfers outside Europe are permitted if: •The destination has an adequacy decision (e.g., Japan, Canada). •Standard Contractual Clauses (SCCs) or Binding Corporate Rules (BCRs) ensure secure handling. •Special circumstances, such as explicit patient consent or urgent public health needs, apply, provided legal safeguards are met.			
Clinical Safety Requirements	6. MHRA Classification **Medical device Class 1 approval** is **essential** for all AVT / Ambient scribing products that use Generative AI for **summarisation**. Companies must NOT extend system capabilities to produce generative diagnoses, **management plans, or other medical calculations without having at least MHRA Class IIa approval.** 			
	7. All software (web-based or desktop installation) MUST be approved by the local governance such as the ICS or Trust, including a review of the DPIA, DCB 0129, including Clinical Safety Case and Hazard Log, and risk mitigations.			

Domain	Requirement	Passed	Fail	Justification
	*<u>For Implementing Institution - Primary or Secondary Care</u> It is essential that a DCB 0160 be created by the implementing institution (as a reflective document to the suppliers DCB0129) before clinical use. This will clarify intended purpose and create local risk control plans that should include establishing a local 'Evolving Technology Test' (post-market surveillance capability) to monitor the performance of the AI scribe and report any adverse events or safety concerns.			
	8. The company must have at least a single <u>embedded</u> Clinical Safety Officer (CSO). External validation is recommended.			
	9. Companies must clearly define the scope of the product (including use cases and additive functionality such as AI language translation) being assured. Subsequent changes to the scope require updates to the DPIA and clinical safety documentation which then needs to be reviewed again before clinical deployment.			
	10. Prompt injection: AVT systems should be protected (guard-railed) to ensure that end-users are NOT giving direct access to an LLM interface and allowed to enter inappropriate instructions or questions. For example. "Calculate the TURNIP score for this patient or suggest the most likely diagnosis"; this is clinical safety requirement ensures appropriate use against intended purpose and mitigates against clinical risks to patients or clinicians than have not been accounted for.			
	11. AVT vendors must demonstrate clinician-in-the-loop validation, provide annotated data on outputs, and maintain a system for continuous validation. *<u>For Implementing Institution - clinical use / clinical responsibility</u>			

Domain	Requirement	Passed	Fail	Justification
	Ambient scribes are a digital tool; it is the clinicians' responsibility to check the summarised AI output and medical accuracy. Accountability for error sits with the clinician.			
	12. AVT vendors must have clear processes for reporting and mitigating adverse events (Error Mitigation and Safety Processes). Inbuilt functionality to support end-user error reporting is advised			
	13. AI Language translation is inherently challenging to quality assure, as language nuances, medical terminology, and contextual accuracy can vary significantly. Unlike traditional clinical processes, human-in-the-loop verification is not feasible at scale, making it impossible for clinicians to reliably validate every AI-generated language translation. Clinicians should not be used as a liability sink for potential inaccuracies, as this would impose an unfair and impractical burden on them. Instead, the responsibility for language translation accuracy must remain with the AVT supplier, ensuring that any AI-provided translations meet regulatory and ethical standards without shifting legal risk onto healthcare professionals.			
Bias and Inclusivity Requirements	14. All AI vendors must disclose the underlying models even if proprietary, to ensure transparency.			
	15. Vendors must provide evidence showing that AI systems have been tested with diverse patient populations and operate without bias.			
Technical Requirements	16. Mandatory integration (either front-end or back-end) with primary or secondary systems is ESSENTIAL to allow for write-back data entry, ensure data provenance, and reduce clinical friction.			
	17. AVT vendors must offer simple voice recognition (VR) or dictation, as well as Ambient AI capability, as a standard feature for system consolidation and to ensure NHS cost effectiveness.			

Domain	Requirement	Passed	Fail	Justification
	18. AVT vendors must provide data on the accuracy of their system routinely, including ‘hallucination rate’ (precision), ‘omission rate’ (recall), and ‘word-error-rate.’			
	19. Systems should be capable of handling multiple consultations across patients (with the ability to add / correct / modify the AVT assisted consultation, before the end of session close) reflecting real-world clinical practice.			
	20. Systems must be adaptable to individual healthcare professionals’ styles, formats, and workflows.			
Business Continuity	21. Offline Functionality & Data Security: systems must support offline audio capture and asynchronous processing, ensuring usability during network outages. If a connection is lost mid-consultation, audio should be locally stored and encrypted, remaining accessible once connectivity is restored to complete clinical documentation. Data Security Measures: • Local encryption ensures recordings are inaccessible to users. • Automatic deletion of recordings within 24 hours or on session logout. • Users must log out after each session to ensure all data is cleared			
Evolving Technology Test	22. Continuous monitoring of accuracy - all vendors must assess ‘DRIFT’ or unintended consequences in performance over time, with formal testing and reporting required on a regular basis (as determined by stability evidence for accuracy).			

NHS T.E.S.T. Section B**. Benefits assessment**

Benefits are assessed across 12 benefit domains with an available score 420 points.

‘Clinical effectiveness, cost-effectiveness and workforce impact are weighted more heavily because they represent critical factors that directly influence the ability to harness technology to provide safer, smarter and kinder care.

Benefit domains

Number	Benefit domain	Points
1	Clinical Effectiveness	90
2	Operational Cost-Effectiveness	60
3	Workforce Impact Assessment	60
4	Integration and Interoperability	35
5	Clinician Experience and Usability	30
6	Training, Adoption, and Human Factors	25
7	Patient Safety and Quality of Care	20
8	Patient Experience and Understanding	20
9	Virtual Care Integration	20
10	Data and Analytics Integration	20
11	What if? - Potential harm analysis	20
12	Environmental and Societal Impact	20

NHS T.E.S.T. Part B – Benefits assessment

Section B - **Benefits assessment / scoring / certification**

#	Focus of Benefit	Sub-Category	Points	Yes	No	Score (and evidence)
1	Patient Safety and Quality of Care	Time for care - does the technology increase time for direct patient care?	15			
		Improved Documentation Accuracy - Evidence to show the technology improves documentation accuracy and completeness.	5			
	Total		**20**			
2	Clinical Effectiveness	Clinical validation - Validated through RCT, clinical trial, or pilot studies in the UK; **studies should be sufficiently powered to show evidence of real-world benefit in the NHS.**	50			
		Care standardisation - Supports improved clinical outcomes through care standardisation.	10			
		Streamline administrative tasks - Reduces the burden of clinical documentation.	10			
		Enhance clinical communication - Improves timeliness of correspondence across care teams.	10			
		Improved coding for diagnostic accuracy - Ensures clearer documentation of patient complexity.	10			
	Total		**90**			

#	Focus of Benefit	Sub-Category	Points	Yes	No	Score (and evidence)
3	Clinician Experience and Usability	Reduced friction of working - Enhances computer-human interaction.	5			
		Faster Documentation - Speeds up and enhances clinical noting.	5			
		Optimised Clinical Workflows - Optimises existing workflows.	5			
		Cognitive load reduction - Reduces task-switching and administrative burden.	10			
		Human Factors improvement - Addresses human factors to mitigate errors.	5			
	Total		**30**			
4	Patient Experience and Understanding	Patient communication and engagement - Improves patient-clinician communication.	10			
		Patient Understanding - Enhances patient understanding of their health.	10			
	Total		**20**			
5	Integration and Interoperability	EHR Integration - Integration with NHS EHR systems across various care settings.	15			
		Interoperability synergy - Enhances the use of existing EHR systems.	10			
		Enhanced EHR clinical narrative - Improves accuracy and consistency of the EHR.	5			

#	Focus of Benefit	Sub-Category	Points	Yes	No	Score (and evidence)
	Total		**30**			
6	Operational Cost-Effectiveness	Economic Impact Assessment - Formal economic evaluation of multi-dimensional benefits.	25			
		Return on Investment (ROI) - Contributes positively to NHS's economic objectives.	10			
		Cost savings with greater functionality - Provides greater capability at a reduced cost.	15			
		Operational savings - Reduces staffing and ICT costs.	10			
	Total		**60**			
7	Workforce Impact Assessment	Support across the NHS workforce - Supports clinical and operational working across NHS staff.	10			
		Effectiveness in multiple clinical settings - Evidence of effectiveness across clinical settings.	10			
		Support across areas of clinical focus - Used across multiple areas of clinical focus.	10			
		Multi-specialty use - Validated across a range of medical, surgical, and allied-health specialties.	10			
		Reduction in staff burnout - Supports staff well-being.	10			
		Enhanced Job satisfaction - Increases clinician job satisfaction.	10			
	Total		**60**			

#	Focus of Benefit	Sub-Category	Points	Yes	No	Score (and evidence)
8	Training, Adoption, and Human Factors	Ease of Use - User-friendly interfaces.	5			
		Technology to support Human Factors - Notifies users of AI vs. human decision-making.	5			
		Personalisation - Tailored to support individual workflows.	5			
		Ease of learning - Ease of adoption and availability of training resources.	5			
		User training and support - Provides clear guidelines for AI outputs.	5			
	Total		**25**			
9	Virtual Care Integration	Primary Care - Optimises workflows in primary care.	5			
		Secondary Care - Expands telemedicine or virtual visit capabilities.	5			
		Ambulance Services - Optimises workflows for telephone care triage.	10			
	Total		**20**			
10	Data &Analytics Integration	Data visualisation – Does the solution (or supplier) include functionality to provide near real-time data visualisation to support clinical care delivery?	10			
		Data-driven decision-making - Does the solution supports data alignment and coordination of care?	10			
	Total		**20**			

#	Focus of Benefit	Sub-Category	Points	Yes	No	Score (and evidence)
11	Disbenfits and Potential harm analysis	Has a disbenefits assessment been completed? This will help anticipate and address disbenefits and foster safer, more equitable technological innovation. Have patients, clinical and operational staff been consulted ?	**10**			
		Explain how this technology (and associated workflows) can fail, or cause harm. Characterise urgency, difficulty and detectability of each harm.	5			
		Explain how these harms might be distributed and mitigated	5			
	Total		**20**			
12	Environmental and Societal Impact	Carbon footprint reduction – Does the technology solution or supplier implement measures to reduce the carbon footprint of this capability (including for example carbon credits)?	5			
		Energy Efficiency - Promotes sustainability by minimising energy consumption.	5			
		Societal Impact - Supports a positive impact on society and/or the NHS.	5			
		Contribution to the UK economy: Is this technology developed by a UK-based business (Sovereign AI) , and does it contribute significantly to UK PLC by bolstering the economy, create jobs, attracting investment, and enhance the UK's global competitiveness in technology-driven industries?	5			
	Total		**20**			

Scoring thresholds

Total Points Awarded	Certification Level	
360 - 410 points	Approved for scale across the NHS (Gold)	
290 - 359 points	Approved for use at a single specified site / workflow (Silver)	
200 - 279 points	Needs Improvement (N/A)	
Below 200 points	NOT recommended for NHS use	

Threshold weighting

Gold Certification is practically impossible without scoring in clinical validation. i.e. 'Has this specific capability been validated through a scientifically powered RCT, clinical trial or pilot studies that should be sufficiently powered to show evidence of real-world benefit in the NHS (50 points).

Silver Certification can still be achieved without 'clinical validation', but it becomes practically impossible to reach silver without approximately 50% of the score from both 'Operational Cost-Effectiveness' and 'Workforce Impact Assessment'.

Use the total score awarded to determine the appropriate certification level. This approach clearly links the overall performance to **specific approval status and provides a systematic way to assess whether the technology is ready for scale or needs further improvement.**

Considerations

Section A - is binary confirming that platforms meet set assurance criteria, but we are mindful that there is a need for benefits scoring approach (Section B) that will evolve in an iterative manner and is currently being validated. As the tool is vendor agnostic, the score produced will also support ranking of different suppliers.